



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سنيه رمضان عبدالمؤمن زيدان
المحافظة : القاهرة
الحى / المدينة : الزيتون
العنوان : ~
رقم البطاقة : 25606221400704 رقم التليفون : 01274038719
العمر (تاريخ الميلاد) : 22-06-1956 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة
قسم العلاج :
الأورام [11129]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سنيه رمضان عبدالمؤمن زيدان National ID: 25606221400704	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٣/٠٦ ٢٥٦.٦٢٢١٤.٠٧.٠٤
 ربة منزل
 أنثى مسلمة أرملة
 البطاقة سارية حتى ٢٠٢٠/٠٦/١٠ ١



جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 منته
 رمضان عبدالمؤمن زيدان
 ٦ ح حسين موسى - سراي القبة
 الزيتون - القاهرة
 ١٩٥٦/٠٦/٢٢
 KA6957315
 ٢٥٦.٦٢٢١٤.٠٧.٠٤



Medical Reports

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ
بسم الله الرحمن الرحيم
الحمد لله رب العالمين
والصلاة والسلام على سيدنا محمد وآله
الطيبين الطاهرين

Patient Information

MRN	2023217166	Name	SAYNAA RAMADAN ABDEL MOAMEN ZEDAN		
ID Type	الرقم القومي	ID Number	25606221400704		
Gender	FEMALE	Nationality	EGYPTIAN	Age	70Year(s)
Written Date	28/07/2026 2:32 pm	Visit Date	27/07/2026		
Diagnosis	[P][C50.0] Malignant neoplasm of nipple and areola				

Report and Recommendation

70-year-old post-menopausal female , HTN , knee OA , metastatic breast cancer at initial presentation to lung & MLN & SC LN.

Lt. breast, Tru cut biopsy 1/2023
Focal invasive duct carcinoma, grade II with major DCIS component

ER: positive 100% (8/8)
PR: Low positive 10 % (3/8)
Her2 -ve.

CT CAP with contrast 21/01/2023
Left breast malignant neoplastic process associated with pathologically enlarged ipsilateral axillary, internal mammary and supraclavicular
as well as mediastinal lymph nodes; metastatic.
Multiple bilateral pulmonary nodules; metastatic.

Bone scan 27/02/2023
No definitive evidence of metastatic osseous deposits.

AI 2/2023 + Zometa for osteoporosis

CT CAP 11/2023
Progressive course of the left axillary, left supra-clavicular and mediastinal lymphadenopathy

shifted to Faslodex 11/2023

CT CAP 6/2026
Mild size progression of the previously noted left supraclavicular lymph nodes.
Manubrium sterni mixed lytic and sclerotic bony lesion with cortical breaching currently
showing more sclerotic texture with intrathoracic soft tissue sheet.
Rather stationary course of the rest of the study.

planned : for continuation of Faslodex , bone scan / approval of CDK4/6i

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ
بسم الله الرحمن الرحيم
الحمد لله رب العالمين
والصلاة والسلام على سيدنا محمد وآله
الطيبين الطاهرين
Medical Department

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ
بسم الله الرحمن الرحيم
الحمد لله رب العالمين
والصلاة والسلام على سيدنا محمد وآله
الطيبين الطاهرين
عيادات طب الأورام
Oncology Clinics

Name: سنيه رمضان عبدالمومن زيدان		
MR. : 9874		
Detailed Medical Report		
		السيد
تحيه طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is currently receiving faslodex and zometa, with laboratory results from Bahyea Labs reported as normal. The patient is also prescribed ibrance 125 mg.		
تحريرا في ,		
	التاريخ :	19-09-2026
	الوقت :	اسماء محمد مصطفى نصر
الطبيب المعالج :		
يعتمد ,		

PATIENT NAME: SAYNAA RAMADAN ABDEL MOAMEN ZEDAN
GENDER: Female
DATE : 22-Jan-2026 09:53 AM

PATIENT ID: 2023217166
AGE: 69 y
ORDER: 3705442,3705443

NON CONTRAST MSCT OF THE CHEST, ABDOMEN AND PELVIS

**** Pathologically proven case of breast cancer.**

****Reviewing the previous study dated 20.10.2025, the current study shows:**

I - CHEST:

- **Rather stationary course regarding the previously noted:**
 1. Left breast heterogenous parenchyma as well as the related parenchymal edema and mild overlying skin thickening with still related calcific foci.
 2. The size and number of the previously noted few enlarged left level I axillary lymph nodes.
- **Size progression of the previously noted:**
 1. Left supraclavicular lymph nodes, the largest measures 14 x 13 mm compared to 8x10 mm along its maximum axial dimensions previously.
 2. One of the left internal mammary lymph nodes measures 12 x 9 mm currently along its maximum axial dimensions compared to 5x7mm previously. Stationary course of the rest of the few prominent bilateral internal mammary lymph nodes.
- **No appreciable time interval changes regarding the previously noted**
 1. Few prominent prevascular and pretracheal lymph node, the former is the largest still measures 9x7mm along its maximum axial dimensions also showing dense matrix calcifications.
 2. Bilateral few pulmonary nodules, the largest is still seen at the left lower lung lobe, lateral basal segment still measures about 6 mm.
 3. Bilateral apical subpleural reticulations, likely of post granulomatous infection sequel.
 4. Middle lobe, lingular and right lower lobar atelectatic bands.
 5. Mild cardiomegaly, for echocardiography correlation.
- **No pleural or pericardial effusion.**

II- ABDOMEN AND PELVIS:

- **No appreciable time interval changes regarding the previously**

PATIENT: SAYNAA RAMADAN ABDEL MOAMEN ZEDAN, 2023217166, Female, 69 y
Report version 3, 25-Jan-2026 09:05 PM

Page 1 of 2

noted:

1. Enlarged liver showing smooth outlines and homogeneous reduced parenchymal attenuation denoting diffuse fatty infiltration with no sizable focal lesions (on non contrast basis).
2. Non-visualized gallbladder with still prominent intra- and extrahepatic bile ducts (post cholecystectomy sequel).
3. Bulky left suprarenal gland with small hypodense fat containing nodule measuring 5 mm, likely myelolipoma.
4. Normal non contrast CT appearance of the pancreas, spleen, kidneys and right adrenal gland.
5. Small fat containing paraumbilical hernia.
6. Dolico- sigmoid colon with no sizable colonic masses.
- Underfilled urinary bladder with no gross masses or diverticulae.
- Prominent left para-aortic lymph nodes.

Examined bones showed stable:

1. D2 and D4 small lytic lesion with related sclerotic line? Hemangiomas.
2. Bilateral femoral tiny sclerotic foci - likely bone islands.
3. L1, L2, S1 and right sacral alum subcentimetric faint sclerotic foci.
4. Manubrium sterni faint sclerotic patch.
5. Spondylo-degenerative changes.

OPINION:

***** Follow up study of a pathologically proven case of breast cancer showing:***

- Size progression of the previously noted:
- 1. Left supraclavicular lymph nodes.
- 2. One of the left internal mammary lymph nodes
- Rather stationary course of the other findings.

Reported By,
Dr , Yomna Ali, MSc
Dr. Ghada Khalifa , MD

25-Jan-2026 09:05 PM

05/25/2026

5

MDT FORM

Name:	زيدة رمضان Zedan	MRN:	9874
Age:	70 Y	ID:	25606221400704
Menopausal Sta		Phone:	01220779533
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve FH
CO Morbidity:	HTN . CHOLECYSTECTOMY	Reason Of MDT:	new case

Clinical Data:

BX:	IDC G 2, WITH MAJOR DICS			
ER:	7/8	PR:	0/8	HER2: +2
				K167: SISH N01

R

L

Sonomammography:

2023
ACR C, LT BREAST EDEMA WITH INCREASED PARENCHYMAL TRABECULATION
AND DIFFUSE THICKENING OF SKIN WITH PATHOLOGICAL AXILLARY LN UP TO
SUPRACLAV

Mets Workup:

*LAST CT CAP SHOWS BIL FEW PULMONARY NODULES
ONE OF LEFT INTERNAL MAMMARY LN
BIL FEMORAL TINY SCLEROTIC FOCI LIKE BONE ISLANDS
RT SACRAL ALUM SCLEROTIC FOCI
MANUBRIUM FAINT SCLEROTIC PATCH
*BONE SCAN : EQUIVOCAL ACTIVE OSSEOUS LESION AT LT STERNO
CLAVICULAR ARTICULATION
UNREMARKABLE REST OF SCAN
T4BN3M1

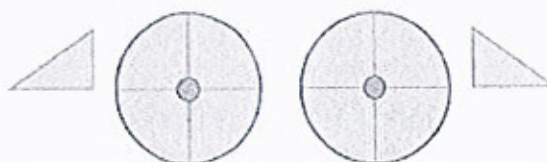
History:

PT WAS DIAGNOSED SINCE 2023 , ON HORMONAL TTT

Recommendations:

MDT Outcome & Plan

For trial of adding CDK4/6 Palbociclib to AI, Close Follow up with CTs and Bone Scan



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نورهان مصطفى	د/ ندى حسن				

2. u_0, v_0, w_0 are

Name : Sania Ramadan Abdel Mo'men Zidan
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 24-04-2026 08:41
Medical No. : 358699
Ins.Company : مُعاهد مستشفى

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

69 year-old female patient with history of left breast cancer, referred for metastatic workup .

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mci of the tracer.

Findings

- Enhanced radiotracer uptake is noticed at the left sterno-clavicular articulation.
- Degenerative changes are noticed at different levels of the spine and around big joints.
- Otherwise uniform radiotracer uptake is noticed allover the surveyed skeleton.

IMPRESSION

- Equivocal active osseous lesion at the left sterno-clavicular articulation for CT correlation to clarify its nature.
- Rest of the scanned skeleton is unremarkable.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

9AVE

Patient Name : الأستاذ / منيه رمضان عبدالمزمن زيدان	Request No. : 13005693
Patient No. : 25606221400704	Request Date : 02/04/2026
Path No. : SR-423/26	Verification Date : 07/04/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 69 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left supraclavicular lymph node in known case of left breast cancer.
Nature of specimen: Referred slides (U/S guided FNA, Cytology).

Gross Examination:

Ten referred slides labelled by (200138-26) were received including five smears, one cell block and four immunostained slides for (ER, PR, Her2 & SISH).

Microscopic Examination:

Cytological examination reveals cellular smears and cell block showing clusters and groups of malignant epithelial cells showing moderately pleomorphic enlarged hyperchromatic nuclei with tendency to tubular formation.

* IPT:

- ER: Moderate nuclear reaction in 80% of tumor cells (score 7/8).
 - PR: Negative nuclear reaction in tumor cells (score 0/8).
 - Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Not amplified.
- (Average Her2 copy No.: (2.35), Her2/CEP17 ratio: (0.979)).

Diagnosis:

Left supraclavicular lymph node, Referred slides (U/S guided FNA, Cytology):

- Metastatic carcinomatous deposits.
- * Immunophenotyping for (GATA-3) is advisable (Paraffin block is needed to perform it).

IPT:

- ER: Positive (score 7/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).

Verified by :

Dr.Duaa Mohamed

Printing Date : 14/05/2026 09:05:40

Page 1 of 3

Printing By : marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / سنية رمضان عبدالمؤمن زيدان	Request No. : 13005693
Patient No. : 25606221400704	Request Date : 02/04/2026
Path No. : SR-423/26	Verification Date : 07/04/2026
Sex : Female	Financial Category : جوستاف رومسي - نفقة الدولة
Age : 69 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- SISH: Not amplified, Group V.
(Average Her2 copy No.: (2.35), Her2/CEP17 ratio: (0.979)).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).

Verified by :

Dr.Duaa Mohamed

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Page 2 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / منيه رمضان عبدالمؤمن زيدان
Patient No. : 25606221400704
Path No. : SR-423/26
Sex : Female
Age : 69 Y
Referring Doc :

Request No. : 13005693
Request Date : 02/04/2026
Verification Date : 07/04/2026
Financial Category : جوسفات رومسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Prof. Dr. Gamal Abd-El-Aty.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 14/05/2026 09:05:40

Page 3 of 3

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